



ICD-0-3

Carcinoma, urothelial
papillary 8/30/13

Site Bladder NOS
C67.9

JW 8/20/13

- SOURCE:
- A. Right ureteral margin (FS)
 - B. Left ureteral margin (FS)
 - C. Left pelvic lymph nodes
 - D. Bladder, resection, partial or total
 - E. Anterior peri-vesicular fat
 - F. Superior common iliac artery lymph nodes (FS)
 - G. Additional left pelvic nodes
 - H. Left common iliac lymph nodes
 - I. Right pelvic lymph nodes
 - J. Final left ureteral margin
 - K. Final right ureteral margin

DIAGNOSIS:

- A. Right ureteral margin, excision: Sections of ureter with no evidence of dysplasia or carcinoma.
- B. Left ureteral margin, excision: Sections of ureter with no evidence of dysplasia or carcinoma.
- C. Left pelvic lymph nodes, excision: Urothelial carcinoma, metastatic to one regional lymph node, with extracapsular extension (1/1).
- D. Bladder, prostate and seminal vesicles, radical cystoprostatectomy:
Bladder:
-Invasive urothelial carcinoma, high grade (WHO grade 3/3), 0.4 cm in diameter.
-Ureteral and urethral margins negative for tumor.
-Polypoid cystitis.
-Non specific granulomatous inflammation.
See comment.
Prostate:
-Urothelial carcinoma, metastatic.
-Benign nodular hyperplasia.
-Chronic inflammation.
Seminal vesicles: No evidence of carcinoma.
- E. Anterior perivesicular fat, excision: Mature fibroadipose tissue.
- F. Superior common iliac artery lymph nodes, excision: Urothelial carcinoma metastatic to two of two regional lymph nodes (2/2).
- G. Left pelvic lymph nodes, excision: Thirteen regional lymph nodes with no evidence of metastatic carcinoma (0/13)
- H. Left common iliac lymph nodes, excision: Urothelial carcinoma metastatic to three of five regional lymph nodes (3/5).
- I. Right pelvic lymph nodes, excision: Urothelial carcinoma metastatic to three of nine regional lymph nodes (3/9).
- J. Final left ureteral margin, excision: Ureter with no evidence of

dysplasia or carcinoma.

K. Final right ureteral margin, excision: Urothelial dysplasia, high grade, no carcinoma identified.

CLINICAL DATA:

The patient is a ■-year-old man with invasive high grade bladder cancer diagnosed on biopsy. He undergoes radical prostatectomy and cystectomy, with biopsies for frozen section.

GROSS DESCRIPTION:

The specimen is received in eleven parts, each labeled with the patient's name and medical record number.

Part A is received fresh and is additionally labeled "#2 right ureteral margin." It consists of one soft, pink-tan, irregular, unoriented tissue fragment, measuring 0.6 x 0.6 x 0.4 cm, that is inked at one end. The specimen is entirely submitted for frozen section diagnosis as FS1, with the frozen section remnant submitted in cassette A1.

Part B is received fresh and is additionally labeled "#3 left ureteral margin." It consists of one soft, pink-tan, irregular, unoriented tissue fragment, measuring 1.3 x 0.8 x 0.5 cm, that is inked at one end. The specimen is entirely submitted for frozen section diagnosis as FS2, with the frozen section remnant submitted in cassette B1.

Part C is received in formalin and is additionally labeled "#1. Left pelvic lymph nodes." It consists of a single piece of yellow-red, fibrofatty soft tissue, measuring 3 x 1 x 0.5 cm. The specimen is bisected and entirely submitted in cassettes C1 and C2.

Part D, additionally labeled "#4 bladder," consists of a radical cystoprostatectomy, including bladder, ureters, prostate and seminal vesicles. The bladder measures 18 x 12 x 4.5 cm, the wall averages 2.5 cm in thickness. The bladder mucosa is pale tan and shows two ulcerations, one in the trigone and one in the dome measuring, respectively, 1.5 and 1 cm in diameter, the bladder wall underlying these areas is indurated and dusky. The surrounding uninvolved mucosa is mostly polypoid. Grossly no tumors are identified in the mucosa and bladder wall. The ureters are patent. The prostate measures 4.5 x 4 x 2.5 cm, the cut surface reveals nodularity of both lobes, no tumor is identified grossly. The right ureter measures 8.5 cm in length and 0.5 cm in diameter. The left ureter measures 10 cm in length and 0.3 cm in diameter. The urethra measures 4.5 cm in length. The right seminal vesicle measures 1 cm x 0.5 cm in average diameter. The left seminal vesicle measures 2 cm x 0.5 cm in average diameter. The posterior surface of the specimen is inked black, anterior-right is inked green, and anterior-left is inked blue. Representative sections are submitted as follows: Cassette D1: Left ureteral margin, inked yellow. Cassette D2: Right ureteral margin, inked yellow. Cassettes D3-D4: Urethral margin from base of prostate. Cassette D5-9: Trigone. Cassette D10: Left prostate. Cassette D11: Left prostate plus seminal vesicle. Cassette D12: Right prostate. Cassette D13: Right prostate plus seminal vesicle. Cassettes D14-D17: Potential lymph nodes. Cassettes D18-21: Trigone ulceration. Cassette D22-24: Dome ulceration. Cassettes D26-28: Nodular area of right prostate. Note: urethra is included in D27-28, which contain a bisected piece.

Part E is additionally labeled "#5 anterior peri-vesical fat." It consists of two pieces of yellow-red fibrofatty tissue, measuring 1 x 0.5 x 0.2 cm in aggregate. The specimen is entirely submitted in cassette E1.

Part F is received fresh and is additionally labeled "#8 superior common iliac artery lymph nodes." It consists of two soft, pink-tan, irregular, unoriented tissue fragments, measuring 4.0 x 2.0 x 0.6 cm in aggregate. A representative section is submitted for frozen section diagnosis as FS3, with the frozen

section remnant submitted in cassette F1. The remainder of the specimen is serially sectioned and submitted in cassette F2.

Part G, additionally labeled "6 - additional left pelvic lymph nodes," consists of multiple red-yellow fragments of soft, fatty tissue, measuring ~ 4.0 x 2.5 x 0.5 cm in aggregate. The largest portion measures ~ 7.0 x 3.0 x 0.5 cm. This latter specimen is cut into thirds. The small fragments are submitted as cassettes G1-G3, and the largest fragment cut into thirds is submitted as cassettes G4-G6.

Part H is additionally labeled "7 - left common iliac lymph nodes." It consists of three portions of red-yellow, fibrofatty tissue, measuring 1.5 x 0.5 x 0.5 cm in aggregate. The specimen is submitted in toto in cassette H1.

Part I, additionally labeled "9 - right pelvic lymph nodes," consists of multiple fragments of red-yellow, fibrofatty tissue, measuring 6.0 x 4.0 x 0.9 cm in aggregate. The specimen is submitted in toto in cassettes I1-I5.

Part J, additionally labeled "10 - final left ureteral margin," consists of a single portion of red-yellow, tubular tissue, measuring 4.0 x 0.8 cm in diameter, with a staple. The staple is removed, and a cross section from the stapled end is submitted as cassette J1. A cross sections from the unstapled side is submitted in cassette J2.

Part K, additionally labeled "11 - final right ureteral margin," consists of two pieces of red-yellow, fibrofatty tissue, measuring 2.0 x 2.0 x 0.5 cm in aggregate, with an attached staple. The staple is removed, and the adjacent cross section is submitted in cassette K1. A representative cross section of the unstapled portion as well as a sample of white-yellow soft tissue are submitted in cassette K2.

COMMENTS:

Bladder Tumor Synoptic Comment 1. Tumor type: Urothelial carcinoma, papillary with squamoid features. 2. Tumor grade: Grade 3/3 (2003 WHO classification). 3. Tumor size: 0.4 cm (D8). 4. Extent of tumor in bladder: 0.4 cm focus of invasive tumor in outer half of the muscularis propria (D8). 5. Lymphatic/vascular invasion: Not present. 6. Epithelial abnormalities in bladder: Polypoid cystitis and nonspecific granulomatous inflammation (post transurethral resection of bladder tumor) see note below. 7. Extension of tumor into organs adjacent to bladder: Prostate. 8. Surgical margins: Urethral margin: Negative. Right ureter margin: High grade urothelial dysplasia. Left ureter margin: Negative. Perivesical margin: Negative. 9. Lymph nodes: Nine of 30 lymph nodes contain metastatic tumor (9/30); the largest metastatic deposit measures 2.0 cm in greatest dimension (C2) ; there is focal extranodal tumor growth (C1). 11. AJCC/UICC stage: pT2bN2MX.

The bladder mucosa, submucosa and muscularis propria of the dome and trigone show an intense nonspecific granulomatous inflammatory reaction with foreign body giant cells and focal necrotizing palisading granulomas (C8). These findings are consistent with the patient's recent history of transurethral resection of his bladder tumor, at these sites. The bladder mucosa is otherwise benign with polypoid type chronic inflammation and focal hyperplasia, with no residual carcinoma or dysplasia. There is a microscopic focus of high grade urothelial carcinoma located in the outer half of the muscularis propria of the trigone. A 0.3 cm focus of high grade urothelial carcinoma is present in the periurethral prostate.

Criteria	Yes	No
Diagnosis Discrepancy		☒
Primary Tumor Site Discrepancy		☒
HIPAA Discrepancy		☒
Prior Malignancy History		☒
Dual/Synchronous Primary Noted		☒
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	Date Reviewed: 8/9/2013	